



ORIGINAL ARTICLE

Mental health nurses' experience of physical health care and health promotion initiatives for people with severe mental illness

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ABSTRACT: Health care for people with severe mental illness is often divided into physical health care and mental health care despite the importance of a holistic approach to caring for the whole person. Mental health nurses have an important role not only in preventing ill health, but also in promoting health, to improve the overall health among people with severe mental illness and to develop a more person-centred, integrated physical and mental health care. Thus, the aim of this study was to describe mental health nurses' experiences of facilitating aspects that promote physical health and support a healthy lifestyle for people with severe mental illness. Interviews were conducted with mental health nurses ($n = 15$), and a qualitative content analysis was used to capture the nurse's experiences. Analysis of the interviews generated three categories: (i) to have a health promotion focus in every encounter, (ii) to support with each person's unique prerequisites in mind and (iii) to take responsibility for health promotion in every level of the organization. The results show the importance of a health promotion focus that permeates the entire organization of mental health care. Shared responsibility for health and health promotion activities should exist at all levels: in the person-centred care in the relation with the patient, embedded in a joint vision within the working unit, and in decisions at management level.

KEY WORDS: health promotion, lifestyle, mental health nursing, person-centred care, severe mental illness.

INTRODUCTION

An important issue for mental health nurses is the prevention of ill health and promotion of health in order to

improve both mental health and physical health (The Swedish Society of Nursing, 2017). It is well known that people with severe mental illness (SMI), such as schizophrenia or other psychotic disorders, have an increased risk of physical ill health than the general population (Crump *et al.* 2013; Olfson *et al.* 2015; Scott & Happell 2011) and this physical ill health is related to a higher risk of developing metabolic syndrome (MetS) (McDaid & Smyth 2015). Contributory factors to this higher risk of MetS, in addition to the antipsychotic medication (Vancampfort *et al.* 2015), are unhealthy lifestyle habits such as sedentary leisure time, low physical activity, poor dietary habits, smoking and alcohol consumption (Heald *et al.* 2017; Scott & Happell 2011).

In general, nurses in mental health care have a positive attitude towards physical health care and

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acknowledge it as a part of their role (Happell *et al.* 2013; Knight *et al.* 2017). Furthermore, mental health nurses appear to have good knowledge of risk factors that increase the risk of physical ill health and how to prevent it; nonetheless, implementation into practice is progressing slowly (Bolton *et al.* 2016). Thus, there might be other issues that impede the promotion of physical health. For example, there is a lack of consensus as to who should have responsibility for the physical health of people with SMI (Ehrlich *et al.* 2014; Wynaden *et al.* 2016), and mental health nurses appear to be ambivalent in their role of promoting physical health (Bradshaw & Pedley 2012; Happell *et al.* 2012). Moreover, physical health is often under-prioritized in mental health care (Gray & Brown 2017). Lack of time (Verhaeghe *et al.* 2013), and lack of continuity and follow-up (Happell *et al.* 2013) are other aspects that have been revealed as barriers to the promotion of physical health in mental health care.

Even though the importance of promoting physical health and improvements in physical health care for people with SMI has been highlighted for decades, there continues to be a lack of integrated physical and mental health care (World Health Organization 2018). Health care for people with SMI is too often separated, and the needs of the whole person are forgotten even though both health care workers and patients view physical and mental health as inseparable (Kristiansen *et al.* 2015). Mental health nurses have an important role in the development of a more person-centred care where physical and mental health are integrated and where health promotion has a dedicated place (Wynaden *et al.* 2016). While prevention of diseases is an important and well-known aspect of the role of mental health nurses, health promotion work must be developed further. Health promotion is defined as 'the process of enabling people to increase control over and to improve their health' (World Health Organization 1986). While prevention focusses on risk factors for ill health and disease, health promotion focusses on factors that can promote health (Tengland 2010). This shift from a pathogenic to a salutogenic view of health can be traced back to Antonovsky's theory of salutogenesis which focusses on resources for health, and the process towards health (Antonovsky 1987). According to Antonovsky (1996), people's salutary factors are important contributors to health promotion research and practice. The previous research on physical health care and implementation of health promotion initiatives in mental health care often focusses on barriers to promoting physical health and healthy living. Therefore,

this study has a salutogenic approach. Thus, the aim of this study was to describe mental health nurses' experiences of facilitating aspects that promote physical health and support a healthy lifestyle for people with SMI.

METHOD

Design

This study has a qualitative design based on interviews with nurses working in psychiatric outpatient clinics with people diagnosed with schizophrenia or other psychotic disorders. An inductive content analysis was used to capture similarities and differences in the nurses' perceptions and experiences (Graneheim & Lundman 2004).

Participants

The participants were recruited from seven psychiatric outpatient clinics in two regions in southern Sweden. Inclusion criteria were that they should be nurses with experience of health promotion initiatives aimed at improving physical health. They should also have experience of supporting a healthy lifestyle for, and together with, people with SMI. First, head of unit was contacted for permission to contact each psychiatric outpatient clinic. In the next step, the managers of the outpatient clinics were contacted, given information about the study, and permission was requested to contact nurses at the specific clinics. Each contacted manager asked the nurses who met the criteria whether they were interested in participating in the present study. Thereafter, contact was made to set a date for an interview. In some cases, participants were recruited by nurses who had already been interviewed, so-called snowball sampling (Polit & Beck 2012). Fifteen nurses, ten women and five men, aged between 30 and 63 years old ($m = 51$), agreed to participate in an interview. Their experience of working in mental health care was 2–45 year ($m = 21$), as a mental health nurse 0–32 years ($m = 14$) and at the current workplace 1–17 years ($m = 6$). All mental health nurses had a Master's degree, or equivalent, or were in education at Master's degree level at the time.

Data collection

The interviews were conducted between May and October 2018. At the request of the participants, all

TABLE 1: *Example of the analyse process for each of the three categories*

Meaning unit	Condensed meaning unit	Code	Subcategory	Category
I think I see a clear connection between whether a person is doing some physical activity... mostly because their social interaction changes and they are more alert of course	A clear connection if a person is doing some physical activity, their social interaction changes and they are more alert	Physical activity promotes the overall health	Promoting a healthy lifestyle	To have a health promotion focus in every encounter
you do not have to be so very good all the time, just good enough... to start moving around a little bit when you have not done it at all is also a very good change...	One does not have to be good all the time, just good enough, to move a little if you have never moved is a good change	To be good enough	Supporting healthy routines at the right level	To support with each person's unique prerequisites in mind
We need as nurse also to be more... maybe talk more about what is the mental health nurse's role, what are we supposed to do and not do, what can we emphasize that needs to be clear in our profession as well, that we need to do, I believe	As mental health nurses we need to talk more about what is our role, what are we supposed to do and not, what can we emphasize that needs to be clear in our profession	Mental health nurses responsibilities in the professional role	In collaboration taking responsibility for the overall health	To take responsibility for health promotion within every level of the organization

fifteen interviews took place at the participant's workplace. All interviews were conducted by the first author. A semi-structured interview guide was used consisting of open questions, with supplementary questions when necessary (Polit & Beck 2012). Each interview started with the question 'What is your view of physical health and living habits, in relation to the person's mental health?' The nurses were then asked to reflect upon opportunities for the patients to manage lifestyle changes and what kind of support they needed. Furthermore, the nurses were asked to reflect upon what facilitates the promotion of physical health and lifestyle changes in their everyday work. The interviews lasted between 36 and 65 min. They were all audio-recorded and transcribed verbatim by the first author.

Ethical approval

The study was approved by the Regional Ethics Committee at Lund University, Sweden, Dnr 2012/267 with an amendment for inclusion of nurses from a nearby region, Dnr 2016/513. Written and oral information about the study, that participation was voluntary, and that the interviews were treated with confidentiality was given by the first author in conjunction with each interview.

Data analysis

Qualitative content analysis inspired by Graneheim and Lundman (2004) was used to analyse the interviews.

First, the interviews were read several times to acquire an understanding of the content as a whole. In the next step, meaning units related to the research question were identified and condensed. Each condensed meaning unit was abstracted and given a code that described the content of each meaning unit. The codes with similar content were reflected upon, abstracted and sorted into categories and subcategories (see Table 1). The strive was to find descriptions close to the text, rather than interpretation of the text. However, some interpretation is required even when the manifest content is analysed (Graneheim, Lindgren, & Lundman 2017). The reflections during the analysis were discussed with the co-authors and on several occasions led to regrouping of the codes and categories. Finally, three categories with associated subcategories were formulated that describe the manifest content (see Table 2).

RESULTS

The categories describe the mental health nurses' perceptions of aspects that were promoting for the patients' health and for a healthy lifestyle, as well as their experiences of aspects that were health-promoting and relevant to their profession and within their organization.

To have a health promotion focus in every encounter

In this category, the nurses' experiences of the importance of a health promotion focus are described and

TABLE 2: Overview of subcategories and categories

Subcategories	Categories
Paying attention to physical health	To have a health promotion focus in every encounter
Promoting a healthy lifestyle	
Follow-up in the short and long term	
Having and sharing the knowledge	To support with each person's unique prerequisites in mind
Supporting healthy routines at the right level	
Promoting and believing in the patient's ability	
Supporting in and towards togetherness	
Having a personal commitment	To take responsibility for health promotion within every level of the organization
Feeling freedom and responsibility in the professional role	
In collaboration taking responsibility for the overall health	

include the subcategories: *paying attention to physical health*, *promoting a healthy lifestyle*, *follow-up in the short and long term* and *having and sharing the knowledge*.

The nurses highlighted the importance of *paying attention to physical health*. The importance of having a clinical gaze to be able to notice bodily changes such as overweight, poor face colour or heavy breathing was emphasized. Nurses often have long-standing contact with patients and know them well, which facilitates the ability to recognize changes in health. Another method is to have regular check-ups such as clinical examinations and blood samples, and to encourage to regular examinations such as mammography. However, the most important aspects were to have a dialogue with the patient about their health. To assure not forgetting important aspects of physical health, checklists were expressed as facilitating.

Conversations about health by using a form about lifestyle habits, and I think it is rather good because it covers a lot... most patients usually appreciate filling it in and then I can sit and discuss with them based on that form (H)

Promoting a healthy lifestyle and motivating the patients were important. The nurses' opinion was that physical health and mental health go hand-in-hand. They could observe positive effects of healthy lifestyle habits, such as physical activity and healthy eating, on

the patients' general health and, in particular, on their mental health. They also pointed out that it could be the other way around, when the patient's mental health was stable, it was easier to achieve physical activity and to eat healthily. However, if the mental health was unstable, the mental health took precedence over the physical health. Physical activity was declared the most important lifestyle habit that affected the patients' health. Physical activity can lead to social interaction, better sleep, better quality of life, less overweight and a reduction in medication.

I think I see a clear connection between whether a person is doing some physical activity (or not) mostly because their social interaction changes and they are more alert, of course (A)

Motivation is a prerequisite for the patient's ability to change lifestyle habits, and the nurses have an important role in motivating the patients. They saw themselves as coaches whose task was to motivate the patient to lifestyle changes and, when necessary, motivating them to accept support.

Follow-up in the short and long term was an important task for the nurses. For example, to check bodily health and to see whether planned actions had been conducted. Another important aim of follow-up was to visualize for the patient how the changes affect daily life.

And I think it is important to follow up and not after a whole year ... yes, it is a little bit of coaching ... your efforts have paid off ... yes ... well done (F)

Follow-up should be routine in every contact with the patients, as well as in a wider context with a longer perspective, for example in healthcare planning, where contributing persons are involved, such as other professionals and next of kin.

Having and sharing knowledge was a prerequisite for having a health promotion focus. The nurses experience that the patients have a varying degree of awareness and understanding about health risks, healthy lifestyle habits and how these affect physical health. The nurses have an advisory and a tutorial function, which includes informing and giving advice to the patients, the next of kin and to other professionals about healthy living and how to facilitate lifestyle changes. To do this requires knowledge on different aspects of health and ill health and the nurses expressed that on occasions their knowledge on how to promote physical health is inadequate.

In a dream world we would have a nurse who works with it (physical health) at the outpatient clinic... because we have so much else to do and then you try to weave it in... take it seriously, because it is a really important part of the person's mental health, not to get diabetes and so... but it is hard to find the time to talk specifically about it (L)

There was a wish for support from people with competence in somatic care. One suggestion was to have a nurse with somatic competence at the outpatient clinic who could focus on physical health.

To support with each person's unique prerequisites in mind

This category consists of the nurses' experiences of the importance of creating support based on each person's unique needs and resources while taking into account the patient's abilities and experiences. The subcategories are *supporting healthy routines at the right level*, *promoting and believing in the patient's ability* and *supporting in and towards togetherness*.

A large part of the nurses' work consists of *supporting healthy routines at the right level*. To have routines in everyday life is a prerequisite for incorporating new habits. The patients struggle to create routines in life; sometimes, it is an effort for the patient just to manage their daily routines. Thus, the changes need to be reasonable. The patients often place high demands on themselves, comparing themselves to other people instead of viewing their own efforts as good enough.

You do not have to be so very good all the time, just good enough... to start moving around a little bit when you have not done it at all is also a very good change... (F)

Lifestyle changes require long-term effort and necessitate well-planned actions and a great deal of patience. It is not unusual for unplanned life events to occur which mean the patient has to put the planned changes on hold. Thus, it is important to wait for the right moment and to be available when the patient is ready to make the changes. The patients require support in setting reasonable goals, which give a greater chance that the changes will become sustainable. Therefore, it is important to support daily routines and to focus on what works so that the demands do not become overwhelming.

I try to find another way and instead talk about what works in life and let that grow... and that includes

physical activity and everything, so I think this is a good way for me to get around it (J)

The changes have to be made stepwise, and the support must be adapted accordingly. The nurses pointed out that it was necessary to lower their own expectations. It is important to let changes take time and to start with small things that can have an impact on everyday life. For example, the importance of activities such as daily walks rather than exercising at the gym was highlighted. The support, the activities and the environment have to be adjusted based on the patient's needs and resources.

Reasonable goals and take one thing at a time, changing one habit over time, to not make too big changes, but find something that you can change, start moving from nothing to ten minutes a day... (G)

The nurses had good experiences of physical activity on prescription as a tool to enable adjusting the activities to the right level. They also highlighted the importance of raising awareness of the goals at an early stage to enable the promotion of healthy routines, because younger patients might find it easier to change lifestyle habits than older patients who have lived longer with their mental ill health.

An important aspect of support in the health promotion work was *promoting and believing in the patient's ability*. The patient's inner strength was an important factor in the health promotion work. Their own wish for change and their belief that they have the capability to change facilitate lifestyle changes. When others initiated the changes, they were not usually sustained.

What I have learned over the years is that, if you are to succeed in making changes it is the person themselves that has to make the changes... but I can be there and provide support (J)

The patient's own willpower is a prerequisite for managing lifestyle changes, and it is important to remember that change must be the patient's own choice. The nurses believed that positive experiences of previous changes in combination with the patient's ability to influence their health could be empowering for their inner strength. Nurses must promote the patient's ability and functionality, and involve them in decisions regarding lifestyle changes.

We, as healthcare professionals, should not take over any functions that they can do, then you actually do the patients a disservice (B)

Being encouraging by showing optimism and conveying hope that change is possible is a solid aspect of support.

Supporting in and towards togetherness was identified as meaningful. Patients who have support in everyday activities, from professionals or someone close to them, have a greater chance of managing and maintaining lifestyle changes than those who are alone. A part of the support therefore consists of getting relatives and friends involved. The nurses revealed the importance of being part of a social context. Loneliness is a barrier to changing lifestyle for many patients, and they need to practice being in social settings. Thus, activities in groups promote health in more than one way. The benefits of being in a group are, in addition to increased knowledge and the chance to share experiences, the opportunity to be in togetherness that may lead to other positive outcomes.

In groups, because then you can see how the patients support each other, the ones who have been in the group for a little longer became a group who... well... and they can exercise together or... some of them start going to the gym together... (I)

Health promotion activities in groups were a part of the nurses' task, and they wanted the opportunity to do so to a much greater extent.

To take responsibility for health promotion within every level of the organization

In this category, personal aspects of the responsibility in the professional role as a nurse were highlighted together with aspects at an organizational level. These comprised the subcategories: *having a personal commitment*, *feeling freedom and responsibility in the professional role* and *in collaboration taking responsibility for the overall health*.

Having a personal commitment was expressed as a prerequisite for promoting physical health and for the patient to receive support with lifestyle changes. Nurses and other healthcare professionals who are interested in this topic and dedicated to working with health promotion probably offer better support.

It is almost as if the patients have a bit of luck or bad luck as to which nurse they see... no offense meant, but we work a little differently (E)

The nurses also pointed out the importance of discussing their own physical health and lifestyle with colleagues, since unhealthy living is not only an issue for

the patients. The nurses need to be acknowledged for their health promotion work, but some nurses shared the experience of being discouraged and being perceived as an inconvenience when trying to introduce health promotion activities for the patients.

Feeling freedom and responsibility in the professional role was important. The nurses' perception was that the physical health of their patients was their responsibility. They needed freedom in their professional role to work with health promotion activities.

It is part of the disease that one cannot manage to fully take care of his or her physical health... being punctual, to take that responsibility, and to eat right, take care of oneself... it is part of the disease so therefore I think it is our responsibility (F)

A more permissive leadership is required. Nurses, as experts in nursing, should be involved in decisions about how their work shall be planned and performed. For example, being given the opportunity to provide support in practical situations when lifestyle changes are accomplished. The nurses have the knowledge and the relation with the patients and thus are best suited for acting as a present support.

I would like to... work more at home with the patients, I think it would be good... there we have no opportunity... and the division with the municipality... but you also see what problems the municipality sometimes has... we who have known the patients for a very long time, we have another relation to the patient for understandable reasons, we have a better alliance which I think would facilitate making changes... (M)

Allocated time, continuity, staff resources and economic resources were expressed as essential for the opportunity to work with health promotion.

In collaboration taking responsibility for the overall health was important to promote physical health and a healthy lifestyle. However, it should be done in collaboration with different caregivers, and it must be clear who is responsible for what. The nurses' experience was that it was difficult to see each other's responsibilities, and therefore, they wanted guidelines that were more precise.

But I believe that the assignment must be very clear and that it should come from the organization, from the managers, and it should be... there are so many different things we should do and, well... so it does not just become a kind of a free choice... but what is included in the assignment (D)

The experience was that health care is still divided into body and mind, and the nurses argued for a more person-centred view of the patient's health. Mental health is prioritized in mental health care, which can result in not detecting signs of physical ill health. Even though medication is important for the patient, the focus on medical treatment is sometimes too strong. The nurses have a coordinating role in the health promotion work. However, being responsible for coordination of the overall health can be a heavy burden and needs to be shared between the various caregivers. The quality of the collaboration between caregivers varies. The nurses emphasized the importance of decisions being taken at an overarching organizational level to increase the collaboration within the organization as well as between the various caregivers. However, the nurses want to have a contributory part in these decisions.

We need as nurses also to be more ... maybe talk more about what is the mental health nurses' role, what are we supposed to do and not do, what can we emphasize that needs to be clear in our profession as well... that we need to do, I believe (K)

It is essential that the entire organization has a health-promoting focus if health promotion is to penetrate through to the care of the whole person. Only then, can care be equal for all patients.

DISCUSSION

Methodological considerations

The purpose of using a qualitative content analysis was to identify variations (Graneheim & Lundman 2004) in the nurses' experiences of promoting physical health and supporting healthy living. To ensure trustworthiness of the study (Lincoln & Guba 1985), the data were based on interviews from fifteen nurses from different outpatient clinic with variations in gender, age and years within the profession. This generated abundant material for analysis as well as a variety of experiences that deepened the understanding of mental health nurses' health promotion work in psychiatric care. The managers of the outpatient clinics asked the nurses for participation in the interviews, and it was unclear how it was performed. However, in the beginning of each interview, this was discussed to ensure that participation was voluntary. Recruitment for the interviews was time-consuming because some outpatient clinics did not respond to the request for a long

time or not at all. Possible reason for this might be a heavy workload or lack of interest. Another limitation was that there might be the nurses who had a specific interest in the topic who agreed to participate. However, since the study has a salutogenic approach with the aim to describe what can stimulate health promotion efforts, the informants were considered to be suitable. Quotations were used to enhance credibility. A challenge during the analysis has been to find descriptions close to text without interpreting too deeply (Graneheim *et al.* 2017). During this process, the analysis has been discussed by all the authors until consensus on the content was reached. All the authors are mental health nurses familiar with the context, and this probably affected the interpretation. However, this pre-understanding can be of value for being able to see the content in its right context.

Reflections on findings

The findings describe various experiences of facilitating aspects for promoting health and supporting a healthy lifestyle, and how health promotion can be managed at different levels, from a personal commitment in the encounter with the patient to decision-making at an organizational level. The nurses' perceptions were that they have responsibility for the overall health of people with SMI. However, knowledge about different aspects of health was revealed as important in enabling a health promotion focus. There was a wish for somatic competence among mental health nurses, which is in line with the need for a more integrated education for nurses to improve the overall health for people with SMI (Martin 2016). Furthermore, even if knowledge of risk factors for physical ill health appears to be good among mental health nurses (Bolton *et al.* 2016), there is a need for education and support if they are to be able to promote health and to provide physical health care in practice (Hennessy & Cocoman 2018). Although the nurses' opinion in the present study was that physical and mental health are integrated, mental health was often prioritized over physical health. This is consistent with results from Small *et al.* (2017) where both service user and mental health professionals revealed that physical health problems were not valued equal to mental health problems. Moreover, the service users experienced limited resources to deal with physical health problems within mental health care despite the fact that integrated care was described as a prerequisite for effective care planning by both service users and mental health professionals (Small *et al.* 2017).

Resources such as inner strength and social support were highlighted in this study to be health-promoting, which is consistent with the salutary factors that Antonovsky (1979) referred to as the Generalized Resistance Resources (GMRs). Being able to identify GMRs and knowing how to use them are important for developing a strong Sense of Coherence (SOC), which is created through positive experiences. Thus, it should be of great importance for nurses in mental health care to support these resources and to take advantage of the patient's experience in the health promotion work. Application of a salutogenic approach in mental health care is previously described by Langeland and Vinje (2017) who highlight social support and positive self-identity as crucial resources for promoting health. Similar findings are emphasized in studies that described the experiences of what enables health for people with SMI (Blomqvist *et al.* 2017). Nurses in the present study stress the importance of group activities not only to increase knowledge about healthy living but as a social contribution that can promote overall health; this is consistent with findings from Park *et al.* (2017) where belonging in a group was emphasized as an influential factor for lifestyle changes.

The findings in this study reveal the importance of a person-centred approach in health promotion work. With a person-centred approach, the care is organized based on each person's needs, resources and capabilities and the patients' narratives are essential if these are to be captured (Ekman *et al.* 2011). The nurses highlight the importance of seeing the person as capable, and only when the patient is recognized as a capable person with abilities is mutual sharing and partnership a possibility. In the current study, the nurses emphasized that lifestyle changes had to be a choice made by the patients. However, Lerbæk *et al.* (2019) argue that there is a risk that this might be a justification for not dealing with physical health issues.

The nurses in this study wanted greater responsibility for promoting health for people with SMI because their expertise and their knowledge and understanding of the patient's situation make them well suited to support health promotion activities. They argue for the possibility to make decisions together with patients that they know would be supportive for a healthy lifestyle without feeling pinioned by organizational restrictions. Organizational factors that can influence the health promotion work are described as, clarification of the responsibility for physical health care, discussion of physical health, lifestyle programme and workload (Happell *et al.* 2014) which are consistent with the

findings in the present study. Promoting physical health and supporting healthy living cannot only be a matter of personal interest. It has to be a mutual understanding within the entire organization and a shared commitment both within the organization and in collaboration with other caregivers. Kristiansen *et al.* (2015) and Small *et al.* (2017) emphasize the importance of clarity around the responsibility for provision of physical health care, a more integrated care and better cooperation between mental and somatic healthcare services. Furthermore, nurses must have the necessary conditions to implement health promotion activities and must be acknowledged by management if health promotion work is to have its given place in mental health care. Thus, there needs to be a health-promoting focus within the entire organization.

CONCLUSION

This study provides an understanding of aspects that are important for mental health nurses in their health promotion work related to physical health and the support of a healthy lifestyle. Having a health promotion focus, supporting with each person's prerequisites in mind and having responsibility for health promotion through the entire organization were aspects that promote the health promotion work in mental health care. Mental health nurses emphasize the importance of having a health promotion focus that permeates the entire organization of mental health care. The shared responsibility for health and health promotion activities should exist at all levels; in a person-centred care in the relation with the patient, embedded in a joint vision within the working unit, and in decisions at management level. Thus, to get a less fragmented care for people with SMI, a changed viewpoint and actions taken at an organizational level are necessary.

Relevance for Clinical Practice

Mental health nurses need knowledge of aspects that promote the health promotion work and facilitate activities that can promote physical health and a healthy lifestyle among people with SMI. Furthermore, the management for mental health care needs to be aware of the nurses' experience of what can create a health-promoting environment for the patients, as well as for the nurses in mental health care. Awareness can be the first step to change an organizational structure to increase the nurse's involvement and influence in

decision-making towards a more integrated person-centred care of the whole person.

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